

Policy on Those Responsible for Governance

«Hospital Name»

Document control

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Prepared by	Medical Superintendent Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

The hospital knows exactly who governs it and what each of those people is answerable for. Governance is a working body that meets, decides and is held to account — not a list of names on a chart.

1. Purpose

This policy explains how «Hospital Name» names the people responsible for governing the hospital, records what each of them is answerable for, and makes sure they actually do the job — setting the hospital's direction, approving its plans and its money, appointing its senior leaders, backing safety and quality work, and telling the public honestly how the hospital is performing.

This policy does not cover patient assessment, clinical care, medication management, patient rights, infection control, or quality and safety monitoring — those are covered in other hospital policies.

2. Scope

This policy applies to the governing body, the person leading the organisation, the Medical Superintendent, department heads, and the Quality Coordinator at «Hospital Name». It also applies to anyone who prepares the plans, budgets, performance reports or public information that governance is asked to approve.

3. Policy standards

«Hospital Name» names in writing the people responsible for governance and what each is answerable for, sets the hospital's vision, mission and values and displays them where people can see them, approves the strategic plan, operational plan and annual budget before the year starts, measures how the hospital is actually performing against its stated mission, appoints its senior leaders, backs safety and quality improvement work with real decisions and real money, builds a clinical governance framework, supports the ethics framework and settles conflicts of interest, and tells the public how its services are performing.

Staff follow the written guidance below, keep the records it requires, and raise anything that is not being done to the Medical Superintendent.

4. Non-negotiable rules

1. Do not leave the governing body undefined. Its members, their roles, and what each is answerable for must be written down before this policy is issued.
2. Do not let authority pass to someone informally. The written document must say when, how, and to whom the governing body's authority can be delegated.
3. Do not let governance meet only when convenient. Meetings happen on a set schedule and minutes are kept every time.
4. Do not treat the vision, mission and values as a website entry. They must be approved, displayed where patients and staff can actually see them, and reviewed — a website page on its own does not meet this.
5. Do not start a financial year without governance having approved the strategic plan, the operational plan and the annual budget, covering both capital and operating spend.
6. Do not present a plan without naming who is responsible for each part and by when.
7. Do not report activity in place of performance. Performance reports go to governance on a regular schedule, preferably quarterly, and must include quality and patient-safety measures against the stated mission.
8. Do not let an action item raised at a governance meeting disappear. It is followed up and closed at a later meeting.
9. Do not let the person running the hospital day to day be appointed by anyone other than those responsible for governance.
10. Do not accept a safety or quality report without acting on it. Where corrective and preventive action is needed, governance allocates the funds and resources.
11. Do not run the hospital without a written clinical governance framework covering patient safety, medication safety, clinical audit, clinical pathways, clinical indicators, education and research.

12. Do not leave an ethical issue or a conflict of interest unresolved past the time frame the ethics framework sets.
13. Do not leave the public without information on the quality and performance of the hospital's services.
14. Staff who see any of these rules broken report it the same shift to the department in-charge or the Medical Superintendent.

5. What we do

5.1 Name the people who govern the hospital, and write down what each is answerable for

«Hospital Name» keeps a written document describing the structure of its governing body — who sits on it, the terms of reference, the by-laws, and what each member is answerable for. Those responsible for governance are accountable for the quality of care, not only for the hospital's finances.

The same document states when, how, and to whom the governing body's authority can be delegated, so nobody has to guess who may decide what. New members may need a formal orientation to what the role involves. Governance meets at set intervals and minutes are kept for every meeting.

The Medical Superintendent keeps this document current and produces it on request. A governing body that exists on paper but never meets does not meet this rule.

5.2 Set the vision, mission and values — and put them where people can see them

Those responsible for governance approve the hospital's vision, mission and values in an authorised document, developed and reviewed together with the hospital's leaders. Input may also be taken from patients, families and the community.

The statement is displayed prominently inside the hospital. Publishing it on the website alone is not enough. It may also be translated and displayed in the local language. The date of approval and of each review is recorded.

5.3 Approve the plans and the budget before the year starts

Those responsible for governance approve the strategic plan, the operational plan and the annual budget. How often the strategic plan is prepared, and in how much detail, depends on the size and type of the hospital — but the operational plan is linked to the strategic plan, and both name who is responsible for each item and the timeframe for achieving it.

The annual budget covers both capital spend and operating spend, and is approved annually. Approval means a dated decision recorded in the minutes — circulating a document is not approving it.

5.4 Check how the hospital is actually performing against its mission

The person heading the organisation prepares performance reports on a regular schedule, preferably quarterly, based on the strategic and operational plans. These reports include quality and patient-safety measures, not activity numbers alone.

Performance is discussed at the governance meeting, decisions are recorded, and action items are followed up at later meetings until they are closed.

5.5 Appoint the senior leaders

Those responsible for governance appoint the hospital's senior leaders. At a minimum, the person responsible for the day-to-day running of the hospital is appointed by governance. That choice may be based on qualifications, training, experience and skills. The appointment decision is recorded in the minutes and supported by a dated appointment letter.

5.6 Back safety and quality work with decisions and money

The minutes of the safety and quality improvement committee are shared with those responsible for governance. What is shared includes the main points on risk management, how risks are reported, any sentinel events, and quality improvement activity.

Governance does not simply receive these reports. Where corrective and preventive action is needed, it allocates the funds and the resources to carry it out, and records that decision.

5.7 Build the clinical governance framework

Those responsible for governance develop a written clinical governance framework. It covers patient safety, medication safety, clinical audit, clinical pathways, clinical indicators, education and research. The framework names who owns it, carries a dated approval, and is reviewed periodically.

5.8 Support the ethics framework and settle conflicts of interest

Those responsible for governance support the hospital's ethical management framework. That means ethical issues are resolved within a defined time frame rather than left open, conflicts of interest are declared and addressed, and research conducted at the hospital is supported to run ethically. Ethics matters raised at governance are recorded along with the decision taken.

5.9 Tell the public how the hospital is performing

«Hospital Name» informs the public about the quality and performance of its services. This can be done through displays, brochures or the website. What is published may include feedback received from patients and other stakeholders — negative as well as positive — the results of surveys carried out by independent third parties, and the results of benchmarking done by professional bodies. Dated samples of what was published are kept.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and properly resourced. Acts as the head of the

	organisation unless the governing body has named someone else.
Medical Superintendent	Owns the day-to-day records and implementation for this policy.
Governing body	Holds the governance duties this policy sets out — identification and roles, vision and mission, approval of plans and budget, performance review, senior appointments, clinical governance, ethics, and public information.
Quality Coordinator	Audits this policy periodically; keeps training records; supports dashboards and committee reviews.
Department heads	Carry out the department-level duties this policy names.

7. Quality monitoring

The Quality Coordinator audits this policy periodically, checking a sample of records against the steps in Section 5, confirming documentary evidence is on file for each asterisked objective element in the sample (marked * in the Level column of Section 11), and confirming the CORE objective element shows no critical gaps.

If an audit finds a gap that is still open after a defined timeframe, the hospital carries out a root-cause analysis. This policy itself is reviewed every year, and sooner if there is a change in the governing body, its terms of reference, or the hospital's vision, mission or values.

8. Training and staff acknowledgement

Staff covered by this policy are trained when they join, and again every year after that. Training covers what is in Section 5 and the non-negotiable rules.

I have read the Policy on Those Responsible for Governance of «Hospital Name». I will follow the processes described.

Name	Designation	Department / floor	Date	Signature

(One row per staff member. The Quality Coordinator holds signed acknowledgements with the induction record.)

9. Distribution

Medical Superintendent; governing body; Quality Coordinator; department heads; staff covered by this policy.

10. Abbreviations

Abbreviation	Meaning
CORE	Core objective element (NABH)
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OE	Objective Element
ROM	Responsibilities of Management (NABH Hospitals 6th Edition chapter)

11. Traceability to NABH HCO Full Accreditation 6th Edition ROM.1

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
ROM.1.a	CORE*	Those responsible for governance are identified and their roles and responsibilities are defined and documented.	Section 3; 5.1	Medical Superintendent
ROM.1.b	Commitment*	Those responsible for governance lay down the organisation's vision, mission and values.	Section 3; 5.2	Medical Superintendent
ROM.1.c	Commitment*	Those responsible for governance approve the documented strategic and operational plans and the organisation's	Section 3; 5.3	Medical Superintendent

		annual budget.		
ROM.1.d	Commitment	Those responsible for governance monitor and measure the performance of the organisation against the stated mission.	Section 3; 5.4	Medical Superintendent
ROM.1.e	Commitment	Those responsible for governance appoint the senior leaders in the organisation.	Section 3; 5.5	Medical Superintendent
ROM.1.f	Commitment*	Those responsible for governance support safety initiatives, clinical governance framework and quality improvement plans.	Section 3; 5.6	Medical Superintendent
ROM.1.g	Commitment	Those responsible for governance shall develop clinical governance framework.	Section 3; 5.7	Medical Superintendent
ROM.1.h	Achievement	Those responsible for governance support the ethical management framework of the organisation.	Section 3; 5.8	Medical Superintendent
ROM.1.i	Commitment	Those responsible for governance inform the public of the quality and performance of services.	Section 3; 5.9	Medical Superintendent

12. Required Records / Evidence Checklist

Records the hospital holds under this policy, listed by objective element.

ROM.1.a — Those responsible for governance are identified and their roles and responsibilities are defined and documented.

- Named list of those responsible for governance, with each member's role and responsibility documented.
- Governance resolution or minute formally identifying the list.
- Dated record of the last review or update of the list.

ROM.1.b — Those responsible for governance lay down the organisation's vision, mission and values.

- Governance-approved vision, mission and values statement, with approval date.
- Evidence of public display — photograph or location log, website page, or induction-pack copy.
- Record of when the statement was last reviewed or reaffirmed.

ROM.1.c — Those responsible for governance approve the documented strategic and operational plans and the organisation's annual budget.

- Governance minutes approving the strategic plan, operational plan and annual budget before the cycle starts.
- The approved strategic plan, operational plan and budget documents themselves.
- Dated resolution or sign-off evidencing approval, not just circulation.

ROM.1.d — Those responsible for governance monitor and measure the performance of the organisation against the stated mission.

- Quarterly performance dashboard pack presented to governance.
- Governance minutes recording what was reviewed and any decision taken.
- Annual performance-review report measured against the stated mission.

ROM.1.e — Those responsible for governance appoint the senior leaders in the organisation.

- Appointment letters for senior leaders, including the person heading the organisation, with dates.
- Governance minutes recording the appointment decision.
- Current reporting-line document naming who each senior leader reports to.

ROM.1.f — Those responsible for governance support safety initiatives, clinical governance framework and quality improvement plans.

- Governance minutes showing safety, clinical-governance or quality-improvement reports were received.
- Budget line or resource allocation record supporting those initiatives.
- Decisions recorded in minutes in response to those reports.

ROM.1.g — Those responsible for governance shall develop clinical governance framework.

- The written clinical-governance framework document (clinical audit, clinical pathways, education, research).
- Dated approval record naming who owns the framework.

- Evidence of periodic review or update of the framework.

ROM.1.h — Those responsible for governance support the ethical management framework of the organisation.

- Governance minutes showing an ethics issue was escalated to and considered by governance.
- Resource allocation record supporting the ethical-management framework.
- Decisions recorded on ethics matters raised.

ROM.1.i — Those responsible for governance inform the public of the quality and performance of services.

- Published quality/performance indicators, accreditation status, or the defined public-information set.
- Dated samples of what was published (display photograph, website snapshot, or notice).
- Record of the feedback channel offered to the public and how it is publicised.

13. References

- NABH Accreditation Standards for Hospitals, 6th Edition — standard ROM.1 (official portal PDF).
- NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition — ROM.1 interpretations.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked [Hospital to define] must be replaced with the hospital's own decision; a document issued with those markers left in place is not an approved policy.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. This policy does not import the Consumer Protection Act, 2019, the Clinical Establishments Act, 2010, or the Mental Healthcare Act, 2017 as a checklist. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

The clinical and technical content reflects recognised national and international guidance current at the date of preparation. «Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed.

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